

Malaria Chemoprophylaxis

Patient Group Direction, version 004, issued 9 September 2026. Three arms: atovaquone/proguanil, doxycycline and mefloquine.

Scope of this PGD

This PGD authorises the supply of malaria chemoprophylaxis for travellers to malarious areas, following a destination risk assessment.

- **PROPHYLAXIS ONLY.** This PGD does not authorise the treatment of malaria in any circumstance. A traveller with fever, whether abroad or within a year of returning, is a medical emergency and must be referred the same day for a blood film.
- **WEIGHT DETERMINES THE DOSE AND THE PRODUCT** in children. Atovaquone/proguanil is supplied in two strengths and the paediatric strength must be used below 40kg. Weigh every child; do not estimate from age.
- The quantity must cover the pre-travel lead-in, the whole time in the malarious area, and the post-travel tail, which differs by agent. See Appendix 2.
- No chemoprophylaxis is completely effective. Bite avoidance is not optional advice and must be given and recorded in every case.
- Destination recommendations change. Use current NaTHNaC / TravelHealthPro country guidance for every consultation; do not work from memory.

Arm 1 , Atovaquone with proguanil

Patient Group Direction for the supply of atovaquone/proguanil for malaria chemoprophylaxis in adults and children weighing 11kg and over.

TWO STRENGTHS. Malarone Paediatric 62.5mg/25mg is used from 11kg to 40kg. The adult 250mg/100mg tablet is used only above 40kg. Supplying the adult tablet to a child in the 11 to 20kg band gives four times the intended atovaquone dose.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to travel health, documented and overseen by the Get Real Health team.
- Must be able to obtain and record an accurate BODY WEIGHT for every child, and to apply the weight bands in Appendix 1. Weight, not age, determines the dose and the presentation.
- Must be competent in destination risk assessment using current NaTHNaC / TravelHealthPro country recommendations, and must not select an agent without doing so.
- Must be able to calculate the full course quantity including the pre-travel lead-in and the post-travel tail, using Appendix 2.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Malaria chemoprophylaxis for travel to an area where atovaquone/proguanil is recommended, in adults and children weighing 11kg and over.
Inclusion criteria	• Weight 11kg or more, measured and recorded. • Travelling to an area where chemoprophylaxis is recommended, established from current NaTHNaC / TravelHealthPro guidance. • Able to take a daily oral dose for the whole course including 7 days after leaving the area. • Valid informed consent given, from a person with parental responsibility where the patient is a child. • No exclusion criterion present.
Exclusion criteria	• No destination risk assessment carried out, or the destination does not require chemoprophylaxis. • Any febrile illness at the time of presentation, or a fever within the last month following travel to a malarious area. Malaria is a medical emergency; refer for the same day. • Treatment of suspected or confirmed malaria. This PGD covers PROPHYLAXIS ONLY. A febrile returning traveller needs urgent hospital assessment and a blood film, not a prophylactic dose. • Pregnancy or breastfeeding. Refer; chemoprophylaxis in pregnancy needs individual specialist assessment and is outside this PGD. • Weight below the minimum for every available agent, or weight not obtainable. • Unable to give valid informed consent, or unwilling to complete the full course including the post-travel tail. • Weight below 11kg. • Known hypersensitivity to atovaquone or proguanil. • Known severe renal impairment, or the patient reports kidney disease or dialysis. Refer. • Taking rifampicin, rifabutin, metoclopramide or tetracycline, which reduce atovaquone concentrations. Refer. • Taking warfarin. Refer for INR monitoring rather than supplying.
Cautions	• Take with food or a milky drink; absorption is materially reduced

	on an empty stomach and this affects protection. • If a dose is vomited within an hour, repeat it. • Advise that this agent is started only 1 to 2 days before travel, so a traveller presenting late can still be covered.
Actions if excluded or declines	Explain why this agent cannot be supplied and whether another arm of this PGD is suitable. Where fever is present or has occurred since travel, refer the same day for urgent assessment and say plainly that malaria must be excluded. Where no agent is suitable, refer to a travel clinic or the GP, and give bite avoidance advice regardless. Document the advice and the decision.

The medicine

Name, form and strength	Atovaquone 250mg with proguanil hydrochloride 100mg tablets (adult strength). Atovaquone 62.5mg with proguanil hydrochloride 25mg tablets (paediatric strength). Both strengths must be stocked to run this arm for children. Confirm which strength is in your hand before supplying.
Legal category	POM.
Route and method	Oral, once daily at the same time each day, with food or a milky drink.
Dose and frequency	11 to 20kg: ONE paediatric tablet (62.5/25mg) once daily. 21 to 30kg: TWO paediatric tablets once daily. 31 to 40kg: THREE paediatric tablets once daily. Over 40kg: ONE adult tablet (250/100mg) once daily. Start 1 to 2 days before entering the malarious area, continue daily throughout, and continue for 7 days after leaving.
Quantity to be supplied	Calculate as: 2 days lead-in + days in the area + 7 days tail. Multiply by the tablets per day for the weight band. Worked example, adult, 14 day trip: 2 + 14 + 7 = 23 days = 23 adult tablets. Worked example, 25kg child, 14 day trip: 23 days at 2 paediatric tablets = 46 paediatric tablets. See Appendix 2. Supply the whole course. A short supply leaves the traveller unprotected at the end, which is when risk is highest.
Maximum treatment period	Continuous use for up to 12 months is supported. Beyond that, refer for specialist advice.
Adverse effects	Common: abdominal pain, nausea, vomiting, diarrhoea, headache. Uncommon: mouth ulcers, rash, raised liver enzymes. Rare: severe skin reactions.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and from whom where the patient is a child. • Patient name, address, date of birth, and the GP with whom they are registered. • BODY WEIGHT in kilograms for every patient under 40kg, and the weight band applied. • Destination, itinerary, departure and return dates, and the source consulted for the destination recommendation. • Which agent was chosen and why, including why any alternative was unsuitable. • Name, form and STRENGTH of the product supplied. Malarone

	and Malarone Paediatric are different strengths and both must be recorded by name. • Date of supply, dose, quantity supplied, and the calculated course length including the tail. • Batch number and expiry date. • Advice given, including bite avoidance and the post-travel fever warning. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday.
Storage	Store below 25C in the original packaging.

Information for the patient

Written information	Supply the patient information leaflet for the strength given, and the Get Real Health bite avoidance and post-travel fever sheet.
Counselling	• Take every day at the same time, with food or a milky drink. Missing doses or taking them on an empty stomach reduces protection. • Keep taking it for 7 days after you leave the malaria area. Stopping when you get home is the commonest reason prophylaxis fails. • No tablet is completely effective. Avoid bites: repellent containing 20 to 30% DEET or 20% picaridin, cover up at dusk and after dark, and sleep under an insecticide-treated net. • If you develop a fever at any time up to a YEAR after returning, seek medical help immediately and tell them you have been to a malaria area. Malaria can kill within 24 hours. • If you are sick within an hour of a dose, take another one.
Follow-up	No routine follow-up. Seek urgent medical attention for any fever during or after travel, and say where you have been.

Arm 2 , Doxycycline

Patient Group Direction for the supply of doxycycline 100mg for malaria chemoprophylaxis in patients aged 12 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to travel health, documented and overseen by the Get Real Health team.
- Must be able to obtain and record an accurate BODY WEIGHT for every child, and to apply the weight bands in Appendix 1. Weight, not age, determines the dose and the presentation.
- Must be competent in destination risk assessment using current NaTHNaC / TravelHealthPro country recommendations, and must not select an agent without doing so.
- Must be able to calculate the full course quantity including the pre-travel lead-in and the post-travel tail, using Appendix 2.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Malaria chemoprophylaxis for travel to an area where doxycycline is recommended, in patients aged 12 years and over.
Inclusion criteria	• Aged 12 years and over. • Travelling to an area where chemoprophylaxis is recommended, established from current NaTHNaC / TravelHealthPro guidance. • Able to take a daily dose for the whole course including 4 weeks after leaving the area. • Valid informed consent given. • No exclusion criterion present.
Exclusion criteria	• No destination risk assessment carried out, or the destination does not require chemoprophylaxis. • Any febrile illness at the time of presentation, or a fever within the last month following travel to a malarious area. Malaria is a medical emergency; refer for the same day. • Treatment of suspected or confirmed malaria. This PGD covers PROPHYLAXIS ONLY. A febrile returning traveller needs urgent hospital assessment and a blood film, not a prophylactic dose. • Pregnancy or breastfeeding. Refer; chemoprophylaxis in pregnancy needs individual specialist assessment and is outside this PGD. • Weight below the minimum for every available agent, or weight not obtainable. • Unable to give valid informed consent, or unwilling to complete the full course including the post-travel tail. • Under 12 years of age. Doxycycline causes permanent tooth discoloration and enamel hypoplasia in younger children. • Known hypersensitivity to doxycycline or other tetracyclines. • Known severe hepatic impairment. • Taking warfarin. Doxycycline potentiates it; refer for INR monitoring. • Taking carbamazepine, phenytoin, phenobarbital or rifampicin, which reduce doxycycline concentrations. Refer. • Taking isotretinoin. Refer. • Systemic lupus erythematosus, myasthenia gravis or porphyria.
Cautions	• Photosensitivity is common and matters in a sunny destination. Advise high-factor sunscreen, covering up and avoiding midday

	sun. - Swallow with plenty of water, sitting or standing, and not immediately before lying down, to avoid oesophageal ulceration. - Separate from antacids, iron and dairy by at least 2 hours. - May increase the risk of vaginal candidiasis. Warn women and consider advising them to carry treatment.
Actions if excluded or declines	As Arm 1.

The medicine

Name, form and strength	Doxycycline 100mg capsules.
Legal category	POM.
Route and method	Oral, once daily, swallowed with plenty of water sitting or standing, and well before lying down.
Dose and frequency	100mg once daily. Start 1 to 2 days before entering the malarious area, continue daily throughout, and continue for 4 WEEKS after leaving.
Quantity to be supplied	Calculate as: 2 days lead-in + days in the area + 28 days tail. Worked example, 14 day trip: 2 + 14 + 28 = 44 capsules. The minimum possible supply for any trip is therefore about 31 capsules. A quantity below 30 cannot be correct for any itinerary.
Maximum treatment period	Continuous use for up to 2 years is supported. Beyond that, refer.
Adverse effects	Common: nausea, diarrhoea, photosensitivity, vaginal candidiasis. Uncommon: oesophageal irritation or ulceration, rash, headache. Rare: benign intracranial hypertension, hepatotoxicity, severe skin reactions.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	- That valid informed consent was given, and from whom where the patient is a child. - Patient name, address, date of birth, and the GP with whom they are registered. BODY WEIGHT in kilograms for every patient under 40kg, and the weight band applied. - Destination, itinerary, departure and return dates, and the source consulted for the destination recommendation. - Which agent was chosen and why, including why any alternative was unsuitable. Name, form and STRENGTH of the product supplied. Malarone and Malarone Paediatric are different strengths and both must be recorded by name. - Date of supply, dose, quantity supplied, and the calculated course length including the tail. - Batch number and expiry date. - Advice given, including bite avoidance and the post-travel fever warning. - Details of any adverse drug reactions and the actions taken. - That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday.
Storage	Store below 25C in the original packaging.

Information for the patient

Written information	Supply the doxycycline patient information leaflet, and the Get Real Health bite avoidance and post-travel fever sheet.
Counselling	● Keep taking it for FOUR WEEKS after you leave the malaria area. This is much longer than some other tablets and it is not optional. ● Take with plenty of water, sitting or standing up, and not just before bed. ● You will burn much more easily in the sun. Use high-factor sunscreen and cover up. ● Avoid indigestion remedies, iron tablets and milk within 2 hours of a dose. ● No tablet is completely effective. Avoid bites as described on your sheet. ● If you develop a fever at any time up to a YEAR after returning, seek help immediately and say where you have been.
Follow-up	As Arm 1.

Arm 3 , Mefloquine

Patient Group Direction for the supply of mefloquine 250mg for malaria chemoprophylaxis in adults and children weighing 5kg and over.

Mefloquine is a weekly agent started 2 to 3 weeks before travel, specifically so that tolerability can be assessed before departure. Do not supply it to a traveller leaving within 2 weeks.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to travel health, documented and overseen by the Get Real Health team.
- Must be able to obtain and record an accurate BODY WEIGHT for every child, and to apply the weight bands in Appendix 1. Weight, not age, determines the dose and the presentation.
- Must be competent in destination risk assessment using current NaTHNaC / TravelHealthPro country recommendations, and must not select an agent without doing so.
- Must be able to calculate the full course quantity including the pre-travel lead-in and the post-travel tail, using Appendix 2.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Malaria chemoprophylaxis for travel to an area where mefloquine is recommended, in adults and children weighing 5kg and over, where there is sufficient time before travel to assess tolerability.
Inclusion criteria	• Weight 5kg or more, measured and recorded. • At least 2 to 3 weeks before departure, so that the first doses can be taken and tolerated before travel. • Travelling to an area where mefloquine is recommended, established from current NaTHNaC / TravelHealthPro guidance. • Valid informed consent given, from a person with parental responsibility where the patient is a child. • No exclusion criterion present.
Exclusion criteria	• No destination risk assessment carried out, or the destination does not require chemoprophylaxis. • Any febrile illness at the time of presentation, or a fever within the last month following travel to a malarious area. Malaria is a medical emergency; refer for the same day. • Treatment of suspected or confirmed malaria. This PGD covers PROPHYLAXIS ONLY. A febrile returning traveller needs urgent hospital assessment and a blood film, not a prophylactic dose. • Pregnancy or breastfeeding. Refer; chemoprophylaxis in pregnancy needs individual specialist assessment and is outside this PGD. • Weight below the minimum for every available agent, or weight not obtainable. • Unable to give valid informed consent, or unwilling to complete the full course including the post-travel tail. • Weight below 5kg. • ANY current or previous psychiatric disorder, including depression, anxiety, psychosis, or a suicide attempt at any time. • Epilepsy or any seizure disorder, or taking an anticonvulsant. • Known hypersensitivity to mefloquine or quinine. • Known cardiac conduction disorder, or a family history of one. • Severe hepatic impairment. • Departing within 2 weeks, so that tolerability cannot be assessed

	before travel. • Occupation requiring fine coordination or spatial discrimination, such as pilots and divers. Refer. • Cardiac conduction disorders, including known QT prolongation, or a family history of either, and concurrent QT-prolonging medicines. Refer. This was an exclusion in version 001 and was lost in the version 002 rewrite; mefloquine carries QT prolongation and arrhythmia in its SPC as rare but serious.
Cautions	• Advise the patient to stop and seek advice if they develop anxiety, depression, restlessness, confusion or unusual behaviour. Neuropsychiatric effects may precede a more serious reaction. • Take with food and plenty of water, on the same day each week. • Prior good tolerance of mefloquine is reassuring, not a reason to exclude.
Actions if excluded or declines	As Arm 1. Where mefloquine is excluded on psychiatric or neurological grounds, note that this is an absolute exclusion and offer an alternative arm rather than seeking a second opinion.

The medicine

Name, form and strength	Mefloquine 250mg tablets.
Legal category	POM.
Route and method	Oral, once weekly on the same day each week, with food and plenty of water.
Dose and frequency	Over 45kg: ONE tablet (250mg) once weekly. 31 to 45kg: THREE QUARTERS of a tablet once weekly. 21 to 30kg: HALF a tablet once weekly. 5 to 20kg: ONE QUARTER of a tablet once weekly. Tablets must be scored to divide accurately. Confirm the product held is scored before supplying a divided dose; if it is not, refer. Start 2 to 3 weeks before travel, continue weekly throughout, and continue for 4 WEEKS after leaving.
Quantity to be supplied	Calculate as: 3 weeks lead-in + weeks in the area + 4 weeks tail, in weekly doses. Worked example, adult, 14 day trip: 3 + 2 + 4 = 9 weekly doses = 9 tablets. The minimum possible supply for any trip is therefore about 8 tablets.
Maximum treatment period	Continuous use for up to 1 year is supported. Beyond that, refer.
Adverse effects	Common: nausea, abdominal pain, diarrhoea, headache, dizziness, sleep disturbance and vivid dreams. Uncommon: anxiety, depression, restlessness, confusion. Rare but serious: psychosis, seizures, suicidal ideation, prolonged vestibular disturbance.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP. Neuropsychiatric reactions in particular should be reported.
Records to be kept	• That valid informed consent was given, and from whom where the patient is a child. • Patient name, address, date of birth, and the GP with whom they are registered. • BODY WEIGHT in kilograms for every patient under 40kg, and the weight band applied. • Destination, itinerary, departure and return dates, and the source

	consulted for the destination recommendation. • Which agent was chosen and why, including why any alternative was unsuitable. • Name, form and STRENGTH of the product supplied. Malarone and Malarone Paediatric are different strengths and both must be recorded by name. • Date of supply, dose, quantity supplied, and the calculated course length including the tail. • Batch number and expiry date. • Advice given, including bite avoidance and the post-travel fever warning. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday.
Storage	Store below 25C in the original packaging.

Information for the patient

Written information	Supply the mefloquine patient information leaflet, the manufacturer alert card where provided, and the Get Real Health bite avoidance and post-travel fever sheet.
Counselling	• Take it on the SAME DAY each week, with food and plenty of water. • STOP TAKING IT and seek advice if you become anxious, low in mood, restless, confused, or feel not yourself. Do not wait to see if it settles. • Vivid dreams and disturbed sleep are common and are not by themselves a reason to stop, but tell us if they are distressing. • Keep taking it for FOUR WEEKS after you leave the malaria area. • No tablet is completely effective. Avoid bites as described on your sheet. • If you develop a fever at any time up to a YEAR after returning, seek help immediately and say where you have been.
Follow-up	Contact the pharmacy if any neuropsychiatric symptom develops before travel, so an alternative can be arranged while there is still time.

Appendix 1 , Atovaquone/proguanil weight bands

Weight, not age, determines the dose AND the product strength. Weigh every child.

11 to 20 kg	ONE Malarone Paediatric tablet (62.5mg/25mg) once daily.
21 to 30 kg	TWO Malarone Paediatric tablets once daily.
31 to 40 kg	THREE Malarone Paediatric tablets once daily.
Over 40 kg	ONE adult tablet (250mg/100mg) once daily.
Under 11 kg	Outside this PGD. Refer.

Appendix 2 , Course length and quantity

Atovaquone/proguanil	- Start 1 to 2 days before entering the area. - Continue 7 DAYS after leaving. - Quantity = (2 + days in area + 7) x tablets per day for the weight band.
Doxycycline	- Start 1 to 2 days before entering the area. - Continue 4 WEEKS after leaving. - Quantity = 2 + days in area + 28 capsules.
Mefloquine	- Start 2 to 3 weeks before travel. - Continue 4 WEEKS after leaving. - Quantity = 3 + weeks in area + 4 weekly doses.

Supply the whole calculated course. The post-travel tail is the part travellers stop early and it is the part that prevents relapse and late presentation.

Appendix 3 , Bite avoidance, to be given in every case

- Insect repellent containing 20 to 30% DEET, or 20% picaridin, applied to exposed skin and reapplied as directed.
- Cover arms and legs from dusk onwards. The Anopheles mosquito bites between dusk and dawn.
- Sleep under an insecticide-treated bed net where accommodation is not screened or air-conditioned.
- Apply sunscreen first, then repellent.

Appendix 4 , The post-travel fever warning

Give this verbally and in writing to every patient.

Any fever occurring during travel, or at any time up to a year after returning from a malarious area, must be assessed urgently and the travel history disclosed. Falciparum malaria can be fatal within 24 hours of the first symptom. No chemoprophylaxis is completely effective, and a traveller who took their tablets correctly can still develop malaria.

Authorisation

Version	v004
Supersedes	Version 003, 9 September 2026
Valid from date	9 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 9 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v004
Date	7 September 2026
Changes	• The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • The cardiac conduction and QT prolongation exclusion is restored to the MEFLOQUINE arm. Version 001 excluded cardiac conduction disorders including QT prolongation and a family history of either; the version 002 rewrite lost it, while mefloquine carries QT prolongation and arrhythmia as rare but serious effects in its SPC. It is restored to that arm only, not to the atovaquone with proguanil or doxycycline arms, where it does not apply. Found by comparing against the archived version 001. • The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • Full clinical review and reissue.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Premises name
 Address
 Postcode:
 ODS code, if applicable

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

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